

FOLLICULAR ROSE GARDEN

TRANSFORMING BURSTING

EXPLODING

WALDENSTRÖM'S MISTY POND

3RD LINE

t(14;18)

3%/YR

BCL2

BR

WATCH

WAIT

ANTIBIOTICS

KILLING BACTERIA

11:18

MALT

HOVERS

HCV

SPLENECTOMY

CRUSHED INTO FIRST

DAA

NODAL

IgM FLARE

SPLIKING UPWARD

11:18

POD24

ENZYME INHIBITOR

TARGET EZH2

TAZEM

MOSUN

MUCOSAL TISSUES

THICK

FILTERING

FILTERING

MARGINAL ZONE HERB PATCH

WALDENSTRÖM'S MISTY POND

>90% MYD88 L265P

1. Follicular lymphoma t(14;18)/BCL2

WHAT YOU SEE

Rose garden, t(14;18) ribbon, BCL2

WHAT IT ENCODES

Follicular lymphoma — most common indolent NHL, t(14;18) overexpressing anti-apoptotic BCL2. Waxing-waning painless adenopathy, bone marrow involvement common, generally incurable but long survival.

BOARD TRAP

FL is indolent and incurable — don't push aggressive chemo for asymptomatic low-burden disease; watchful waiting is correct.

2. Watchful waiting

WHAT YOU SEE

Man on bench, WAIT

WHAT IT ENCODES

Asymptomatic, low-tumor-burden follicular lymphoma is observed (watch-and-wait) — early treatment doesn't improve survival. Treat for symptoms, cytopenias, organ compromise, or high bulk (GELF criteria).

BOARD TRAP

Treating asymptomatic low-burden FL doesn't prolong life — observation is the evidence-based answer.

3. Transformation to DLBCL

WHAT YOU SEE

Transforming/bursting tree, exploding

WHAT IT ENCODES

~3%/year risk of histologic transformation of FL to aggressive DLBCL — signaled by rapid nodal growth, rising LDH, B symptoms, or a focally hot PET. Requires rebiopsy and aggressive therapy.

BOARD TRAP

A sudden LDH spike + rapid node growth in known FL = transformation — rebiopsy and treat as DLBCL, not as indolent flare.

4. ~3%/year transformation rate

WHAT YOU SEE

3%/YR sign

WHAT IT ENCODES

The annual transformation rate of follicular lymphoma is roughly 3% per year, cumulative over time. Transformed disease worsens prognosis and changes treatment intent.

BOARD TRAP

Don't ignore new constitutional symptoms in long-standing FL — the steady transformation risk makes vigilance essential.

5. Rituximab (anti-CD20)

WHAT YOU SEE

Antibody, target CD20

WHAT IT ENCODES

Rituximab (anti-CD20 monoclonal) is the backbone for indolent B-NHL — alone as maintenance or with chemo (BR, R-CHOP). Screen HBV before use due to reactivation risk.

BOARD TRAP

Rituximab targets CD20+ B-cells only — and mandates HBV screening to prevent fatal reactivation.

6. EZH2 inhibitor (tazemetostat)

WHAT YOU SEE

Enzyme inhibitor, target EZH2

WHAT IT ENCODES

Tazemetostat, an EZH2 inhibitor, is approved for relapsed follicular lymphoma — especially EZH2-mutant tumors. Reflects targeted/epigenetic therapy in relapsed indolent disease.

BOARD TRAP

EZH2 inhibition is for relapsed/refractory FL — not first-line; check for EZH2 mutation to predict best response.

7. Gastric MALT — H. pylori

WHAT YOU SEE

Watering can killing bacteria, MALT

WHAT IT ENCODES

Gastric MALT (extranodal marginal zone) lymphoma is H. pylori-driven; early-stage regresses with eradication antibiotics alone. The most common extranodal marginal zone site.

BOARD TRAP

Antibiotic eradication is first-line for early gastric MALT — but t(11;18)+ tumors won't respond and need radiation/systemic therapy.

8. MALT t(11;18)

WHAT YOU SEE

MALT herb, 11;18

WHAT IT ENCODES

t(11;18) (API2-MALT1, NF-κB activation) predicts H. pylori eradication failure in gastric MALT — these need local radiation or systemic therapy instead.

BOARD TRAP

t(11;18) = antibiotic-resistant MALT; eradication therapy alone is inadequate.

9. Splenic marginal zone

WHAT YOU SEE

Spleen, DAM (crushed)

WHAT IT ENCODES

Splenic marginal zone lymphoma — massive splenomegaly, villous lymphocytes, cytopenias from hypersplenism, frequent HCV association. Treat HCV if present; otherwise splenectomy or rituximab.

BOARD TRAP

HCV-positive splenic MZL may regress with antiviral therapy — treat the virus before reaching for chemo.

10. Nodal marginal zone

WHAT YOU SEE

Nodal herb patch

WHAT IT ENCODES

Nodal marginal zone lymphoma is the least common MZL — indolent, treated like follicular lymphoma (observation or rituximab-based therapy when symptomatic).

BOARD TRAP

Nodal MZL behaves like FL; it lacks the H. pylori (gastric) or HCV/spleen (splenic) associations of the other MZLs.

11. Waldenström — MYD88 L265P

WHAT YOU SEE

Misty pond tombstone, >90% MYD88 L265P

WHAT IT ENCODES

Waldenström macroglobulinemia (lymphoplasmacytic lymphoma) — IgM monoclonal gammopathy with >90% carrying MYD88 L265P. Causes hyperviscosity, anemia, neuropathy.

BOARD TRAP

MYD88 L265P + IgM paraprotein = Waldenström; absence of MYD88 should prompt reconsidering the diagnosis.

12. IgM hyperviscosity

WHAT YOU SEE

IgM FLARE vials spiking upward

WHAT IT ENCODES

Large pentameric IgM causes hyperviscosity syndrome — headache, visual changes, mucosal bleeding, neuro symptoms. Plasmapheresis is the emergency treatment to rapidly drop IgM.

BOARD TRAP

For symptomatic hyperviscosity, do plasmapheresis FIRST; also beware rituximab-induced IgM 'flare' worsening viscosity — pre-treat.

13. BTK inhibitor (ibrutinib)

WHAT YOU SEE

Knight in pond (BTK)

WHAT IT ENCODES

BTK inhibitors (ibrutinib, zanubrutinib) are highly effective in MYD88-mutant Waldenström, acting downstream of MYD88 signaling. CXCR4 mutation can reduce response.

BOARD TRAP

BTK inhibitors work best in MYD88-mutant Waldenström; CXCR4 co-mutation blunts response and delays it.
